

ANNEX A: Draft Section-by-Section Legislative Outline

Universal Catastrophic Coverage with Health Savings Empowerment Act

Title I — Definitions and General Provisions

Sec. 101. Definitions.

Establishes statutory definitions for terms used throughout the Act, including “catastrophic plan,” “eligible individual,” “lawfully present resident,” “universal catastrophic credit,” “routine care,” “annual HSA allocation,” “chronic-condition funding track,” “ultra–high-cost therapies,” “rural hospital,” “rural access-capacity payment,” “temporary rural implementation floor,” “catastrophic threshold,” “federal reinsurance offset,” “HSA administrator,” “supplemental coverage,” “rural household access supplement,” “behavioral-health institutional provider,” and “provider-impact stabilization.”

Sec. 102. Rule of Construction; Preservation of Private Comprehensive Coverage.

Provides that nothing in this Act shall be construed to prohibit or restrict the offering, sale, or purchase of private comprehensive health insurance; specifies that the federal framework established by this Act pertains to financing and tax treatment of catastrophic protection and HSAs, not to the legality of broader private coverage.

Sec. 103. Hyde-Style and Conscience Protections.

Prohibits the use of federal catastrophic credits, federal HSA deposits, and federal reinsurance payments for abortion services except in cases of rape, incest, or where the life of the mother is endangered, consistent with existing federal law; affirms that no individual, employer, insurer, or health-care entity shall be required, as a condition of participation in programs under this Act, to provide, pay for, or refer for services contrary to sincerely held religious or moral convictions.

Title II — Universal Catastrophic Coverage (UCC)

Sec. 201. Establishment of Universal Catastrophic Coverage Standard.

Creates a national catastrophic coverage standard for eligible individuals, specifying minimum actuarial characteristics, benefits above the catastrophic threshold, and uniform federal rules for qualifying catastrophic plans.

Sec. 202. Covered Benefits Above the Catastrophic Threshold.

Defines covered catastrophic benefits to include hospital inpatient care, emergency and trauma services, major surgery, intensive care, and other high-severity medical events that occur above the national catastrophic deductible, as specified by the Secretary of Health and Human Services.

Sec. 203. National Catastrophic Deductible and Indexation.

Sets the initial national catastrophic deductible and family threshold by statute and establishes a formula for annual indexation. The Secretary shall publish annual indexed amounts, apply rounding rules, and report to Congress on deductible growth, household exposure, and actuarial effects. Any material change outside the statutory indexation formula requires congressional authorization or an automatic adjustment mechanism under Title XI.

Sec. 204. Ultra–High-Cost Therapy Exclusion and Coordination.

Provides that ultra–high-cost gene therapies and orphan biologics are not financed through UCC catastrophic credits or reinsurance under this Act but remain eligible for coverage and payment under existing Medicare, Medicaid aged/blind/disabled categories, and designated orphan-drug or rare-disease carve-out mechanisms, as determined by statute or subsequent regulation.

Sec. 205. Optional State-Administered Public Catastrophic Plan.

Authorizes states to operate a state-administered catastrophic plan that adheres to the federal catastrophic standard, deductible, and reinsurance parameters; such plans may receive federal catastrophic credits on behalf of eligible enrollees but may not offer subsidized benefits that reduce the effective catastrophic threshold.

Title III — Health Savings Empowerment (HSE)

Sec. 301. Universal Eligibility for Health Savings Accounts.

Establishes eligibility for HSAs for all eligible individuals enrolled in UCC-compliant catastrophic plans, notwithstanding prior-law high-deductible health plan requirements; designates qualified HSA administrators and custodial rules.

Sec. 302. Annual HSA Funding and Allocation Rules.

Creates a federal HSA funding structure providing base deposits, income-indexed deposits, pediatric deposits, chronic-condition supplements, and rural household access supplements where applicable. Pediatric deposits are set by age band and indexed annually to preserve real purchasing power. Exact statutory amounts are specified in the Act and reflected in the scoring tables.

Sec. 303. Definition and Treatment of Routine Care.

Defines “routine care” as services with predictable utilization and low financial variance, including primary care visits, standard laboratory testing, low-complexity imaging, routine specialist visits, preventive services, common outpatient behavioral-health services, and chronic-condition management services; specifies that routine care is ordinarily paid directly from HSAs and is not covered under the catastrophic benefit.

Sec. 304. Direct Primary Care Compatibility.

Confirms that HSA funds may be used to pay for direct primary care arrangements and that such arrangements do not constitute insurance for purposes of this Act; clarifies interaction with catastrophic coverage requirements.

Sec. 305. HSA Guardrails and Permitted Uses.

Specifies qualifying medical expenses, restrictions on non-medical withdrawals, penalties for misuse, rollover rules, and reporting obligations for HSA custodians; authorizes the Secretary of the Treasury to issue regulations governing compliance and data reporting.

Sec. 306. Rural Household Access Supplement.

Authorizes additional HSA deposits for eligible households residing in designated rural, frontier, medically underserved, or low-density areas. Funds are limited to HSE-eligible routine, chronic-care, telehealth, behavioral-health, monitoring, and access-related medical services and may not be used to purchase supplemental insurance riders, reduce the catastrophic deductible, finance comprehensive wraparound coverage, or subsidize nonmedical consumption.

Title IV — Employer Exclusion Phase-Out and Wage Adjustment

Sec. 401. Ten-Year Phase-Out of Employer Tax Exclusion for Health Benefits.

Provides for a staged reduction of the exclusion from taxable income for employer-sponsored health benefits over ten years, with a fixed annual percentage step-down schedule, and directs additional wage income to be subject to standard income and payroll taxation.

Sec. 402. Preservation of Existing Collective Bargaining Agreements.

Allows multi-year collective bargaining agreements in effect as of the date of enactment to maintain existing health-benefit structures until the expiration of the contract term; applies the new tax treatment prospectively at renewal.

Sec. 403. Employer Contributions to HSAs During Transition.

Permits employers to make HSA contributions on a tax-favored basis during the transition period, subject to annual caps and nondiscrimination rules; clarifies interaction between employer contributions, the universal catastrophic credit, and the phase-out of the exclusion.

Title V — Federal Reinsurance Architecture

Sec. 501. Establishment of a Federal Catastrophic Reinsurance Layer.

Creates a federal reinsurance program that pays a defined share, such as 80 percent, of allowed costs above a specified attachment point, such as \$50,000 per enrollee per year, with parameters set by the Secretary based on actuarial analysis and subject to statutory budget constraints.

Sec. 502. State Coordination and Waiver Integration.

Authorizes coordination with ACA section 1332 State Innovation Waivers and SSA section 1115 Medicaid demonstration authority, subject to statutory limits, federal approval, budget-neutrality requirements where applicable, and the UCC/HSE catastrophic standard. Section 1332 pass-through funding may be coordinated with catastrophic-plan stabilization, HSA support, or state-level transition assistance where permitted by law. Section 1115 authority remains limited to Medicaid demonstrations and may be used only for Medicaid-related transition, chronic-care, rural-access, or population-specific coordination approved under existing Medicaid demonstration standards.

Sec. 503. Administrative Requirements and Reporting.

Sets reporting, auditing, and transparency requirements for reinsurance payments; mandates annual public reports on reinsurance performance, geographic distribution of claims, and fiscal impact relative to projected outlays.

Title VI — Medicaid LTSS, ABD, and Medically Complex Populations

Sec. 601. Protection of Medicaid Long-Term Services and Supports (LTSS) and Aged, Blind, and Disabled (ABD) Populations.

States that eligibility, benefits, and financing for Medicaid LTSS and ABD populations are unaffected by this Act, except for coordination provisions explicitly authorized elsewhere; affirms that these populations remain under existing federal-state Medicaid structures.

Sec. 602. Transition of Medicaid Expansion Adults into UCC/HSE.

Provides for a structured transition of ACA Medicaid expansion adults into UCC catastrophic coverage combined with HSE accounts, with

state-level implementation options and federal guidance on timing and coordination.

Sec. 603. Protection for Medically Complex Children.

Clarifies that medically complex children currently served under Medicaid waiver programs or specialized categories remain eligible for those programs and that UCC/HSE pediatric provisions and chronic-condition funding tracks are coordinated with, and do not diminish, such coverage.

Sec. 604. Pediatric HSE Coordination and Pediatric Provider Protection.

Coordinates pediatric HSA deposits with CHIP, Medicaid pediatric coverage, and medically complex children's programs. Clarifies that pediatric HSE support supplements, and does not displace, existing CHIP or Medicaid protections. Requires provider-impact monitoring for children's hospitals and pediatric specialty providers during implementation.

Title VII — Chronic-Condition and Behavioral-Health Funding Track

Sec. 701. Chronic Spending Benchmark and Classification.

Defines criteria for classification into the chronic-condition funding track based on multi-year expenditure patterns and clinical criteria, using data sources such as the CMS Chronic Conditions Warehouse; delegates methodological detail to the Secretary of Health and Human Services.

Sec. 702. Enhanced HSA Seeding for Chronic Conditions.

Sets statutory minimum HSA deposits for enrollees meeting chronic-condition criteria, calibrated to median disease-management costs and indexed to preserve real value over time.

Sec. 703. Condition-Based Care Bundles.

Authorizes the use of condition-based annual bundles for chronic care management, including diabetes, COPD, heart failure, oncology

monitoring, behavioral-health continuity, addiction-treatment follow-up, and other chronic-condition categories, with fixed-price arrangements and reporting standards for quality and cost outcomes.

Sec. 704. Income-Protection Cap for Chronic Out-of-Pocket Liability.

Establishes a cap on net out-of-pocket spending for certified chronic-disease management services below the catastrophic threshold, with parameters linked to income and expenditure benchmarks; specifies that the catastrophic plan design remains uniform and that adjustments occur through financing, not benefit variation.

Sec. 705. Behavioral-Health Continuity and Institutional Stabilization.

Authorizes behavioral-health routine-care support, HSA-compatible outpatient behavioral-health services, and institutional behavioral-health stabilization for qualifying psychiatric and behavioral-health providers identified through provider-impact analysis. Stabilization support is provider-side, bounded, and tied to access preservation, transparency, and reporting requirements.

Title VIII — Eligibility, Verification, and Program Integrity

Sec. 801. Eligibility for Catastrophic Credits and Federal HSA Deposits.

Defines eligible individuals as:

1. U.S. citizens;
2. lawful permanent residents; and
3. long-term lawfully present non-citizens who have maintained lawful presence for at least five years and filed federal income tax returns for at least three of the preceding five years.

Explicitly excludes individuals in unlawful immigration status, asylum applicants, humanitarian parolees, temporary visitors, and other

categories not made eligible by statute from federal catastrophic credits, federal HSA deposits, and reinsurance-funded benefits under this Act.

Sec. 802. Eligibility Verification Using Federal Data Systems.

Requires the use of IRS, SSA, and DHS data to verify eligibility for catastrophic credits and federal HSA deposits; authorizes data-matching procedures and establishes standards for identity resolution, error correction, appeal, and administrative review. Self-attestation alone is insufficient to establish eligibility for federal catastrophic credits, federal HSA deposits, or reinsurance-funded benefits.

Sec. 803. Program Integrity, Anti-Fraud Mechanisms, and Penalties.

Provides for GAO and OIG audits of program operations, defines civil and criminal penalties for fraudulent claims or misuse of HSA funds, and authorizes the Secretary of the Treasury and the Secretary of Health and Human Services to suspend or recoup payments where fraud or abuse is identified.

Sec. 804. Emergency-Care Noninterference.

Provides that nothing in this Act alters obligations under EMTALA or other applicable federal emergency-care requirements. Ineligibility for federal catastrophic credits, federal HSA deposits, or reinsurance subsidies does not alter hospital obligations to provide emergency screening and stabilizing treatment where required under existing law.

Title IX — Rural Health Stability and Access Protection

Sec. 901. Rural Access-Capacity Payment.

Establishes a permanent rural access-capacity payment for qualifying rural, frontier, Critical Access Hospital, and other designated low-volume access providers. The payment equals 2 percent of net patient revenue, capped at \$5 million per qualifying provider, subject to eligibility, reporting, and access criteria established by the Secretary.

Sec. 902. Temporary Rural Implementation Floor.

Provides that, during the implementation period, no qualifying rural

provider receiving the rural access-capacity payment shall experience a modeled UCC/HSE transition impact below -1.5 percent of net patient revenue after application of the permanent rural access-capacity payment. The implementation floor is temporary, reviewable, and limited to transition-period access protection.

Sec. 903. Rural Eligibility, Reporting, and Guardrails.

Defines eligibility for rural access protection using objective geographic, low-volume, access-necessity, and provider-class criteria. Requires participating providers to comply with price transparency, HCRIS-compatible reporting, access monitoring, and restrictions on the use of stabilization support. Payments may not be used to reduce patient deductibles, finance supplemental insurance wraparound coverage, preserve opaque facility-fee billing, subsidize unrelated expansion, or increase commercial rates.

Sec. 904. Rural Virtual Access Network and Chronic-Care Bundle Integration.

Authorizes qualifying rural providers to participate in rural virtual access networks, telehealth-supported specialist arrangements, and HSA-purchasable chronic-care bundles. Establishes standards for local originating-site participation, care coordination, telehealth integration, advance price disclosure, and quality reporting.

Sec. 905. Rural Transformation Track.

Creates a voluntary, time-limited rural transformation pathway for qualifying providers receiving stabilization support. Eligible uses include transparent price-posting infrastructure, telehealth network integration, chronic-care bundle development, HCRIS-compatible reporting improvements, care-management redesign, workforce cross-training, diagnostic and laboratory access coordination, and service-line planning for low-volume inpatient units.

Sec. 906. Coordination with Medicare Critical Access Hospital Rules.

Provides that nothing in this Act diminishes Medicare Critical Access

Hospital protections, rural hospital payment formulas, Sole Community Hospital rules, Medicare-dependent hospital provisions, or related Medicare rural payment structures. UCC/HSE rural access payments complement, and do not replace, existing Medicare rural authorities.

Sec. 907. Rural Access Monitoring and Closure Review.

Requires state and federal access monitoring for rural and frontier providers receiving stabilization support. A closure or major service-line reduction attributed to UCC/HSE implementation triggers access review when the provider serves a low-density area, is the only acute-care facility within a defined travel-time radius, or provides essential emergency, obstetric, inpatient, or diagnostic capacity.

Title X — Supplemental Coverage Guardrails

Sec. 1001. Definition of Supplemental Coverage.

Defines supplemental coverage as limited-benefit products that wrap around UCC catastrophic plans, including dental and vision benefits, defined long-term care riders, disease-specific riders, and specific drug carve-outs that do not materially alter the catastrophic structure.

Sec. 1002. Prohibition on Effective Deductible Reduction.

Prohibits any supplemental product from materially reducing the effective catastrophic deductible or replicating first-dollar comprehensive coverage; disqualifies plans that do so from receiving any federal catastrophic credit or favorable tax treatment under this Act.

Sec. 1003. Transparency and Disclosure Requirements for Supplemental Products.

Requires clear disclosure of supplemental-plan premiums, covered services, actuarial value, administrative load, and interactions with UCC catastrophic plans; mandates standardized consumer-facing forms and periodic reporting to federal regulators.

Title XI — Fiscal Caps, Cost Projections, and Automatic Correction Mechanisms

Sec. 1101. Federal Outlay Cap and Benchmark.

Establishes a statutory cap on combined federal outlays for catastrophic credits, federal HSA deposits, and reinsurance as a percentage of projected outlays under the ACA subsidy and cost-sharing framework, using CBO baseline projections as the benchmark.

Sec. 1102. Automatic Adjustment Mechanism.

Provides that if total federal outlays under this Act exceed the statutory cap beyond a specified tolerance band, automatic adjustments shall occur in the subsequent year to the catastrophic deductible index and/or HSA deposit levels, subject to defined floors and ceilings, in order to restore alignment with the cap. Automatic adjustments may not reduce statutory minimum HSA deposits for low-income households, pediatric deposits, chronic-condition supplements, rural household access supplements, behavioral-health continuity support, or rural provider access-capacity payments below protected statutory floors unless Congress affirmatively amends those floors.

Sec. 1103. Annual Budgetary and Program Performance Reporting.

Requires CBO-style and MedPAC-style annual reports to Congress detailing federal outlays, savings relative to ACA, distributional impacts, provider-impact stabilization, rural access effects, behavioral-health institutional effects, and performance of automatic correction mechanisms.

Title XII — Transition Implementation and Exchange Wind-Down**Sec. 1201. Phase-Out of ACA Exchanges and Individual Market Subsidies (Years 1–3).**

Provides for a three-year scheduled phase-out of ACA premium tax credits and cost-sharing reductions, with gradual replacement by catastrophic credits and HSE deposits; prohibits new exchange subsidy entitlements after the final transition year.

Sec. 1202. Transition of Medicaid Expansion Adults to UCC/HSE (Year 2).

Specifies the timing and process by which Medicaid expansion adults are migrated into UCC catastrophic coverage combined with HSE funding, with federal guidance for states and temporary financial support during the transition.

Sec. 1203. Employer Exclusion Phase-Out (Years 1–10).

Coordinates implementation of the employer exclusion phase-down with Titles III and IV; sets out the year-by-year schedule and reporting requirements to Congress on wage and coverage effects.

Sec. 1204. No-Lapse Coverage Guarantee (Years 1–3).

Provides that no eligible individual who had qualifying coverage under ACA exchanges, employer-sponsored insurance, or Medicaid expansion as of the date of enactment shall experience a lapse in catastrophic coverage during the first three transition years, provided that such individual complies with enrollment procedures established under this Act.

Sec. 1205. CHIP and Pediatric Continuity Clause.

Ensures that children enrolled in CHIP or in Medicaid pediatric coverage retain continuous coverage during the transition; coordinates pediatric HSA deposits with existing programs and prohibits administrative actions that create coverage gaps for children.

Sec. 1206. State Implementation Flexibilities and Regulatory Authority.

Authorizes states to sequence their own UCC/HSE implementation within federal time frames, consistent with Titles II through XI; grants the Secretaries of Health and Human Services and the Treasury authority to issue regulations, guidance, and temporary waivers necessary to effectuate an orderly transition.

U.S. CODE INTERACTION SUMMARY

Indicative Drafting Structure for the Universal Catastrophic Coverage and Health Savings Empowerment Act — 119th Congress

This Annex identifies the primary areas of federal law that would require amendment, supplementation, or cross-reference to implement the Universal Catastrophic Coverage and Health Savings Empowerment (UCC/HSE) framework. Citations refer to existing statutory authorities; final numbering and amendatory language will be assigned by the Office of Legislative Counsel.

I. Internal Revenue Code (Title 26)

A. Health Savings Accounts — 26 U.S.C. §223

Amend §223 to:

1. Extend HSA eligibility to all individuals enrolled in a federally qualified catastrophic plan.
2. Authorize federal deposits, including base, income-indexed, pediatric, chronic-condition, and rural household access supplements.
3. Establish authorized uses and penalties for misuse.
4. Standardize custodial and administrative rules across Treasury-approved institutions.

B. Employer-Sponsored Insurance Exclusion — 26 U.S.C. §106

Amend §106 to:

1. Establish the statutory schedule for phased reduction of the employer exclusion.
2. Preserve pre-existing collective bargaining agreements through expiration.
3. Clarify treatment of employer HSA contributions during the transitional period.
4. Coordinate with §§105, 125, and 162(l) as necessary for employer compliance.

C. Premium Tax Credits — 26 U.S.C. §36B

Amend §36B to:

1. Sunset ACA premium tax credits consistent with the exchange wind-down schedule.
2. Redirect eligible individuals to the universal catastrophic credit and federal HSA deposits.
3. Retire reconciliation and income-verification provisions tied to marketplace enrollment.

II. Public Health Service Act / Affordable Care Act (Title 42)

A. Abortion Funding Restrictions & Conscience Protections

- Section 1303 of the ACA — 42 U.S.C. §18023 (Special Rules)
- Church Amendments — 42 U.S.C. §300a-7

Amend to:

1. Apply Hyde-consistent restrictions to catastrophic credits, federal reinsurance payments, and federal HSA deposits.
2. Maintain that federal funds may not be used for abortion except in cases of rape, incest, or life endangerment.
3. Preserve existing conscience protections for providers, insurers, and plan administrators.

B. ACA Exchange Provisions — 42 U.S.C. §§18031, 18071 (ACA §§1311, 1402)

Amend to:

1. Establish the federal schedule for marketplace wind-down.
2. Eliminate cost-sharing reduction provisions.
3. Retire marketplace data reporting, enrollment machinery, and actuarial-value requirements.
4. Provide for the orderly transition of enrollees into UCC catastrophic coverage.

C. State Innovation Waivers — 42 U.S.C. §18052 (ACA §1332)

Clarify that:

1. States may coordinate existing §1332 pass-through funding with catastrophic-plan stabilization, HSA support, state-level transition assistance, or rural access support where permitted by law.
2. §1332 waivers remain available but must conform to UCC/HSE's federal catastrophic standard.
3. §1332 authority does not permit states to waive the uniform national catastrophic core, recreate federally subsidized comprehensive first-dollar coverage, or use pass-through funds in a manner inconsistent with UCC/HSE fiscal guardrails.

III. Social Security Act (SSA Titles XVIII, XIX, XXI)

A. Medicaid — SSA §§1902–1905 (42 U.S.C. §§1396a–1396d)

Amend to:

1. Transition ACA expansion adults into UCC/HSE within the implementation period.
2. Preserve Medicaid LTSS, ABD, medically fragile children's benefits, and HCBS waiver authorities.
3. Clarify FMAP treatment for states during the coverage transition.
4. Coordinate acute-care catastrophic payments with state Medicaid systems.

B. Medicaid Demonstrations — SSA §1115 (42 U.S.C. §1315)

Clarify that:

1. §1115 demonstration authority remains available for state-designed chronic-care bundles, Medicaid-related rural innovations, Medicaid transition supports, and population-specific coordination.

2. §1115 authority remains limited to Medicaid demonstrations and does not create general-purpose funding for non-Medicaid UCC/HSE populations.
3. UCC catastrophic standards cannot be waived except as explicitly permitted by Congress.

C. CHIP — Title XXI (42 U.S.C. §1397aa et seq.)

Specify that:

1. CHIP eligibility, benefits, and financing remain unchanged.
2. CHIP enrollment is not disrupted during ACA exchange wind-down or Medicaid redeterminations.
3. Pediatric catastrophic continuity is guaranteed.
4. Pediatric HSE deposits coordinate with, and do not replace, CHIP or Medicaid pediatric coverage.

D. Medicare — Title XVIII (SSA §§1801 et seq.)

Clarify that:

1. Ultra-high-cost therapies, including gene therapies and certain orphan biologics, remain financed under Medicare, Medicaid ABD, or orphan-drug authorities.
2. No shift of such therapies into the catastrophic pool.
3. UCC/HSE does not modify Medicare benefits, eligibility, Medicare Advantage, Part D, Medigap, or existing Medicare payment structures.

E. Rural Hospital Framework — SSA §1820 (42 U.S.C. §1395i-4)

Clarify that:

1. UCC/HSE rural stabilization measures complement, and do not replace, the Critical Access Hospital authority.

2. The permanent rural access-capacity payment and temporary implementation-period floor coordinate with existing Medicare rural payment protections.
3. UCC/HSE does not diminish Medicare Critical Access Hospital reimbursement, rural hospital flexibility authorities, Sole Community Hospital rules, Medicare-dependent hospital provisions, or other existing Medicare rural payment structures.
4. Rural access support applies to the UCC/HSE transition and non-Medicare replacement population; Medicare rural payment rules remain intact.

IV. ERISA (Title 29) — Employer Plans & Preemption

A. ERISA §514 Preemption — 29 U.S.C. §1144

Specify that:

1. Federal catastrophic-plan definitions, supplemental-coverage guardrails, and prohibitions on federally subsidized first-dollar wraparound coverage apply uniformly to employer-sponsored plans, ERISA plans, non-ERISA group plans, and individual-market products to the extent provided by federal law.
2. ERISA preemption does not displace federal definitions of catastrophic coverage, routine-care exclusions, supplemental-coverage limitations, or tax-favored HSA rules established under this Act.
3. Employers may continue to offer supplemental benefits that comply with federal actuarial, tax, and disclosure limits.
4. Employer contributions remain subject to the employer-exclusion phase-down, HSA contribution limits, and supplemental-coverage guardrails established under this Act.

V. Immigration and Nationality Act (INA) — Eligibility Verification

To implement UCC/HSE eligibility rules, cross-reference:

- 8 U.S.C. §1101 et seq. (immigration definitions)
- 8 U.S.C. §1158 (asylum)
- 8 U.S.C. §1182(d)(5) (parole)
- 8 U.S.C. §§1225, 1227 (inspection, inadmissibility, and removal-related authorities)

Statutory intent:

1. Eligible: U.S. citizens, lawful permanent residents, and long-term lawfully present residents meeting statutory residency and tax-filing requirements.
2. Ineligible: individuals without lawful status, asylum applicants, humanitarian parolees, temporary entrants, and other categories not made eligible by statute.
3. Verification through cross-matched IRS, SSA, and DHS data; no eligibility based on self-attestation alone.
4. Emergency-care obligations under EMTALA and other federal emergency-care requirements remain unchanged.

VI. New Statutory Units

Placement Determined by Legislative Counsel

A. Universal Catastrophic Coverage Standard

Create a new statutory part establishing:

- The national catastrophic deductible.
- Covered catastrophic services.
- Eligibility for the universal catastrophic credit.
- Insurer certification standards.
- Federal reinsurance structure and cost-sharing rules.
- Coordination with state-administered public catastrophic plans.

B. Health Savings Empowerment

Create authority for:

- Federal HSA deposit formulas, including base, income-indexed, pediatric, chronic-condition, and rural household access supplements.
- Coordination rules with IRS and Treasury.
- A statutory definition of “routine care” not eligible for insurance coverage under the catastrophic benefit.
- Allowable uses, rollovers, and anti-abuse provisions.
- Direct primary care compatibility.

C. Chronic-Condition and Behavioral-Health Funding Track

Create:

- Chronic-condition eligibility benchmarks.
- Enhanced HSA deposits for qualifying conditions.
- Income-based household protection caps.
- Condition-based care bundle authority.
- Behavioral-health continuity support.
- Institutional behavioral-health stabilization for qualifying psychiatric and behavioral-health providers.
- Coordination with UCC catastrophic rules.

D. Eligibility & Verification Mechanisms

Create:

- Citizenship and lawful-presence eligibility rules.
- Explicit exclusions for ineligible categories.
- Mandatory DHS/SSA/IRS verification procedures.
- Error-correction, appeal, and administrative-review processes.
- Emergency-care noninterference provisions.

E. Rural Health Stability and Access Protection

Create:

- Permanent rural access-capacity payment equal to 2 percent of net patient revenue, capped at \$5 million per qualifying rural provider.
- Temporary implementation-period floor of -1.5 percent of net patient revenue after application of the access-capacity payment.
- Eligibility criteria for rural, frontier, Critical Access Hospital, and other low-volume access providers.
- Rural transparency, reporting, and access-monitoring requirements.
- Rural Virtual Access Network authority.
- Rural chronic-care bundle integration.
- Rural Transformation Track.
- Rural access monitoring and closure-review procedures.
- Coordination with Medicare Critical Access Hospital rules under SSA §1820.

F. Pediatric HSE and Pediatric Provider Coordination

Create:

- Pediatric HSA deposit authority by age band.
- Coordination with CHIP and Medicaid pediatric coverage.
- Protection for medically complex children.
- Pediatric provider-impact monitoring.
- Children's hospital and pediatric specialty-provider stabilization where identified through provider-impact analysis.

G. Fiscal Guardrails & Automatic Adjustments

Create:

- Federal outlay cap benchmarked to the ACA baseline.
- Automatic adjustment mechanism for deductible indexing and HSA deposit formulas.
- Protected statutory floors for low-income HSA deposits, pediatric deposits, chronic-condition supplements, rural household access supplements, behavioral-health continuity support, and rural provider access-capacity payments.

- Annual Treasury/HHS reports, reviewed by CBO and MedPAC.

H. Transition & Exchange Sunset

Create:

- ACA exchange wind-down timeline.
- Medicaid expansion transition sequence.
- No-lapse coverage guarantee.
- CHIP and pediatric continuity clause.
- State implementation flexibilities and federal funding coordination.

VII. Committee Jurisdiction

119th Congress

House

- **Ways & Means:** IRC amendments, premium credits, HSA structure, employer exclusion, tax treatment of supplemental coverage.
- **Energy & Commerce:** Medicaid, CHIP, catastrophic standards, rural health, provider-impact stabilization, insurance market rules, public health provisions.
- **Education & Workforce:** ERISA-based employer plan issues, employer-sponsored coverage transition, workplace benefit interactions.

Senate

- **Finance:** IRC amendments, Medicare, Medicaid, CHIP, federal reinsurance architecture, HSA financing, rural access-capacity payments.
- **HELP:** PHSA provisions, insurance markets, consumer protections, provider regulation, behavioral-health access, rural telehealth and access standards.

VIII. Purpose of This Annex

This Annex serves as a non-exhaustive drafting guide for the Office of Legislative Counsel, committee counsels, CBO analysts, Treasury/HHS rule writers, and appropriations reviewers. It identifies the statutory anchors required to construct UCC/HSE without prescribing final numbering or amendatory text. Final statutory placement, cross-references, conforming amendments, and effective-date language remain subject to Legislative Counsel review.